

1. Administrative & Study Identification

Full Study Title: A Phase 1 Study of PF-08046052/SGN-EGFRd2 in Advanced Solid Tumors **Short Title/Acronym:** Study of PF-08046052/SGN-EGFRd2 **Protocol Number:** SGNEGFRd2-001 (Alias: C5841001, NCI-2023-08886) **NCT Number:** NCT05983133 **Sponsor Name:** Seagen Inc. (a wholly owned subsidiary of Pfizer) **Investigational Product:** PF-08046052 / SGN-EGFRd2 (Bispecific Gamma Delta-T-Cell Engager Directed to EGFR) **Phase of Development:** Phase 1 **Medical Monitor:** Pfizer / Seagen Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety and tolerability of PF-08046052/SGN-EGFRd2 and determine the maximum tolerated dose (MTD) or recommended phase 2 dose (RP2D) in participants with advanced solid tumors. **Secondary Objectives:** To evaluate the preliminary antitumor activity (including ORR and PFS), pharmacokinetics (C_{trough}, AUC, T_{max}), and immunogenicity (antidrug antibodies) of the investigational product.

2.2 Background & Rationale To address the significant unmet medical need in patients with relapsed, refractory, or standard-therapy-intolerant advanced solid tumors. Epidermal growth factor receptor (EGFR) is over-expressed in multiple solid tumor types. PF-08046052/SGN-EGFRd2 is a novel bispecific Gamma Delta-T-Cell engager designed to direct T-cells to specifically target and kill EGFR-expressing cancer cells, offering a potential new therapeutic avenue for these refractory conditions.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Intervention Model:** Single Group Assignment **Blinding:** Open-label (None) **Randomization:** N/A (Sequential assignment for dose escalation, optimization, and expansion parts) **Primary Purpose:** Treatment

3.2 Planned Enrollment & Duration Target \$N\$: Approximately 275 participants (across Parts A, B, and C) **Number of Sites:** Global (Including United States and United Kingdom) **Estimated Study Start:** 14-Nov-2023 (Actual) **Estimated Primary Completion:** 14-Aug-2026 **Estimated Study Completion:** 14-Aug-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants must have a histologically- or cytologically confirmed metastatic or unresectable solid malignancy from one of the following tumor types: Colorectal cancer (CRC), Non-small cell lung cancer (NSCLC), Head and neck squamous cell cancer (HNSCC, non-nasopharyngeal subtype ONLY), or Pancreatic ductal adenocarcinoma (PDAC). 2. Disease that is relapsed, refractory, or intolerant to standard of care therapies. 3. Eastern Cooperative Oncology Group (ECOG) Performance Status score of 0 or 1. 4. Age: 18 years and older. 5. Sex: All genders. 6. Healthy Volunteers: No.

4.2 Exclusion Criteria 1. History of another malignancy within 3 years before the first dose of study treatment, or evidence of residual disease. 2. Known active central nervous system metastases or leptomeningeal disease. 3. Treatment with an aminobisphosphonate IV within 4 weeks of the first dose, or history of thromboembolic phenomena within 6 months prior to the first dose (or contraindication to prophylaxis).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: SGN-EGFRd2 administered in 3- or 4-week cycles (e.g., Q1W, Q2W, Q3W, or Q4W scheduling) during the dose escalation, optimization, and expansion phases. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, or subject withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care therapies as clinically indicated. **Prohibited:** Intravenous aminobisphosphonates; concurrent investigational agents; and corticosteroids for brain metastases unless discontinued at least 7 days prior to the first dose.

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :---
| :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, Medical History, Baseline Scans, Archival/Fresh Tumor Biopsy | | **Baseline** | Day 0 | First Dose, 16-hour inpatient monitoring post-dose for safety/tolerability | | **Interim** | Every Cycle | Safety Labs, PK/ADA sampling, Efficacy Assessment (tumor scans) | | **End of Study** | Up to 3 Years Post-Dose | Final AE tracking (up to 90 days post last dose), Survival Follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Number of participants with adverse events (AEs) evaluated through 90 days after last study treatment, up to approximately 1 year. **Measurement Method:** Clinical investigator assessment of incidence and severity of AEs graded per CTCAE criteria.

7.2 Secondary & Exploratory Endpoints * Objective Response Rate (ORR) defined per RECIST 1.1. * Progression-free survival (PFS) up to approximately 2 years. * Pharmacokinetic parameters including \$C_{trough}\$, \$AUC\$, and \$T_{max}\$. * Number of participants developing antidrug antibodies (ADAs).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Patient monitoring for up to 16 hours following the first dose. AEs continuously tracked up to 90 days post-discontinuation. **Serious Adverse Events (SAEs):** Routine 24-hour reporting threshold for expedited regulatory review. **Data Monitoring Committee (DMC):** Internal Safety Dose Review Committee guiding safety and clearance for escalation into subsequent cohorts in Part A and Part B.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Analysis Set (all patients who receive at least one dose) and Efficacy Evaluable Population (patients with measurable baseline disease and ≥ 1 post-baseline scan). **Sample Size Justification:** A target $N \approx 275$ is planned, driven by algorithmic dose-escalation requirements (Part A: ~ 75), dose optimization (Part B: ~ 40), and expansion (Part C: ~ 160). **Handling of Missing Data:** PK/ADA parameters missing are not imputed; time-to-event outcomes censored at last known alive/progression-free date.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote source data verification by sponsor monitors. **Audit Trail:** Validated eCRF tracking with electronic signatures, audit trails, and data clarification logs for query resolutions.

11. Study Identifiers & Governance

Primary ID: SGNEGFRd2-001 (Alias: C5841001, NCI-2023-08886)
Registry Number: NCT05983133 **Sponsor/Lead Organization:** Seagen Inc. (a wholly owned subsidiary of Pfizer) **Study Title:** Study of PF-08046052/SGN-EGFRd2 in Advanced Solid Tumors **Official Regulatory Title:** A Phase 1 Study of PF-08046052/SGN-EGFRd2 in Advanced Solid Tumors

12. Clinical Framework (Oncology Specific)

Primary Condition: Relapsed/Refractory Advanced Solid Tumors (CRC, NSCLC, HNSCC, PDAC) **Diagnosis Threshold:** Histologically/cytologically confirmed unresectable locally advanced or metastatic disease. **Medical Methodology:** Bispecific Gamma Delta-T-Cell Engager Therapy **Optimization Target:** EGFR-targeting and localized immune effector (T-cell) activation

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical trial pathway involving dose escalation followed by dose expansion. **Education Protocol (HCP):** Protocol specific training, AE (such as CRS) management, and dose-escalation management training. **Education Protocol (Patient):** Informed consent outlining required biopsies, intense monitoring after the first dose, and potential side effects. **Quality Audit Mechanism:** Clinical Data Management source data verification and centralized medical review of treatment safety endpoints.

14. Observation & Follow-Up Schedule

Total Duration: Up to 3 years after the last participant gets their last dose **Onsite Visit Cadence:** Every cycle (3-4 weeks) **Remote Monitoring Frequency:** Every 4 weeks (Q4W) or quarterly during off-treatment survival follow-up **Checklist Review Interval:** Tumor imaging scans evaluated every 12 weeks (Q12W)

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				